



Social support and isolation in narcolepsy and idiopathic hypersomnia: An international survey

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ABSTRACT

Objective: To evaluate factors associated with feeling supported in adjusting to life when diagnosed with narcolepsy or idiopathic hypersomnia.

Methods: An online survey was disseminated by nonprofit organization Project Sleep via email and social networks. Intended recipients were adults (≥ 18 years old) diagnosed with narcolepsy or idiopathic hypersomnia. Multivariable regression was used to assess predictors of feeling supported in adjusting to life with these conditions.

Results: The survey was completed by 1308 individuals (narcolepsy type 1, 48.4 %; narcolepsy type 2, 28.7 %; idiopathic hypersomnia, 22.9 %). Overall, 68.7 % of respondents were ≤ 40 years of age; 87.7 % were female; and 88.8 % were White. Respondents lived in 38 different countries (65.5 % in the United States). At time of diagnosis, most (91.1 %) did not know anyone with the same diagnosis. At time of the survey, 486 respondents (37.2 %) still did not know anyone with their diagnosis. Only 32.3 % of respondents felt supported in adjusting to life with narcolepsy or idiopathic hypersomnia. Knowing someone with the same diagnosis at the time of the survey was a significant predictor of perceived support. Younger age (18–30 years) and being married were also associated with greater perceived support, whereas a diagnosis of narcolepsy type 2 or idiopathic hypersomnia (versus narcolepsy type 1) and living outside the US were associated with less perceived support.

Conclusions: People with narcolepsy or idiopathic hypersomnia often do not know others living with the same condition, even though this is associated with feeling supported. Clinicians should assess for social support and assist with resource identification.

1. Introduction

Central disorders of hypersomnolence—narcolepsy type 1, narcolepsy type 2, and idiopathic hypersomnia—are conditions unified by the predominant symptom of excessive daytime sleepiness [1]. Narcolepsy is a chronic neurological disorder that affects approximately 1 in 2000 people [2]. Narcolepsy type 1 is typically characterized by the pentad of symptoms including excessive daytime sleepiness, cataplexy, sleep paralysis, sleep-related hallucinations, and disrupted nighttime sleep [2]. Cataplexy is the most specific symptom of narcolepsy type 1 and is

usually associated with low levels of the neuropeptide hypocretin (also called orexin), which can be measured in cerebrospinal fluid (CSF) [3]. Most typically, cataplexy in adults manifests as episodes of sudden muscle weakness triggered by emotions such as laughter, anger, or surprise [3]. Narcolepsy type 2 is characterized by excessive daytime sleepiness, an absence of cataplexy, and CSF hypocretin levels in the normal range [4]. Medication therapy for narcolepsy is usually lifelong, and polypharmacy is frequently needed to help manage symptoms [5].

Idiopathic hypersomnia is a chronic sleep disorder involving persistent sleepiness without the short latency to REM sleep that

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characterizes narcolepsy [1]. In addition to excessive daytime sleepiness, symptoms experienced by people with idiopathic hypersomnia may include daily sleep amounts of 10 h or more and extreme sleep inertia, difficulties waking up with alarm clocks, and feeling groggy for long periods of time [6,7]. Additionally, naps are frequently unrefreshing [6,7]. Idiopathic hypersomnia is diagnosed when a person displays symptoms for at least 3 months and other causes of excessive daytime sleepiness have been excluded [1]. The prevalence of idiopathic hypersomnia is unknown, with inconsistent findings across studies [6]. A recent investigation in a community sample estimated the prevalence of idiopathic hypersomnia as 1.5 % using ICSD-3 criteria, which suggests that this disorder may be more common than previously recognized [8].

Both narcolepsy and idiopathic hypersomnia are chronic conditions that can have serious effects on social, occupational, and emotional functioning [9,10]. Public understanding of narcolepsy is limited and often inaccurate, with media often portraying the condition as humorous rather than identifying it as a serious condition requiring medical attention [11]. Further, symptoms are intermittent and often imperceptible to an outsider. As a result, people with narcolepsy often face stigma and are labeled by teachers, coworkers, supervisors, and others as antisocial, lazy, careless, unmotivated, or faking [12]. People with narcolepsy report feeling socially isolated (even within their own families), inferior to others, and hesitant to disclose their disorder, fearing consequences and reactions they may receive [13,14]. Misperceptions and low awareness of narcolepsy also perpetuate delays in timely detection and diagnosis [11,15]. Studies report a median age of onset of 16 years and an approximate delay in diagnosis of 8–15 years [9]. It is estimated that up to 50 % of people with narcolepsy remain undiagnosed [2].

Narcolepsy detrimentally affects health-related quality of life (QoL) due to difficulties in managing the constellation of symptoms and associated psychosocial challenges, exacerbated by social isolation and stigmatization [13–16]. Importantly, education, recreation, driving, and relationships are frequently affected [16,17]. In addition to such disease-related impacts, people with narcolepsy experience higher levels of unemployment and disability benefits claims in comparison with the general population [18–20]. Symptoms of depression are commonly reported by people with narcolepsy [21], and health-related stigma was identified as a key contributor to depression in a study of young adults with narcolepsy [14]. Moreover, disease impact on relationships leads to fewer people with narcolepsy being married or cohabiting [19,22]. This, in turn, contributes to fewer social support mechanisms.

Idiopathic hypersomnia also has a wide range of detrimental effects. The disorder is not well understood—the term “idiopathic” indicates that the underlying pathophysiology is unknown [6]. Unfortunately, the term idiopathic is often interpreted negatively, suggesting idiopathic hypersomnia is not a real disease but a type of sleepiness that many people experience on occasion. Impacts of idiopathic hypersomnia include societal misperceptions and stigmatization, cognitive difficulties, and decreased work productivity [10]. Idiopathic hypersomnia interferes with participation in social activities, which contributes to feelings of social isolation [10]. Furthermore, many people with idiopathic hypersomnia report symptoms of anxiety and/or depression [10]. Because idiopathic hypersomnia is a diagnosis of exclusion and has symptoms that overlap with other sleep and psychiatric disorders, it may be challenging for clinicians to recognize, especially when associated with depression or anxiety, leading to delays in diagnosis [23].

Despite extensive documentation of factors that negatively affect health-related QoL in people with narcolepsy or idiopathic hypersomnia, supportive factors have not been well researched. In this study, we evaluated associations between knowing someone living with the same condition and feeling supported in adjusting to life with narcolepsy or idiopathic hypersomnia. This improved characterization of social support, and its impact on adjusting to diagnosis, addresses an important knowledge gap to inform future clinical guidelines and advance patient-centered care models.

2. Methods

2.1. Study design, participants, and data collection

An online survey was conducted by Project Sleep, a nonprofit organization dedicated to raising awareness about sleep health, sleep equity, and sleep disorders. The survey was conducted between March 28 and April 11, 2021. Intended recipients were adults (≥ 18 years of age) diagnosed with narcolepsy type 1, narcolepsy type 2, or idiopathic hypersomnia. Initially, the survey was sent to individuals who were part of the Project Sleep community (<https://project-sleep.com/>). The survey link was sent via email to Project Sleep's Listserv and was posted on Project Sleep's social media accounts (ie, Facebook, Twitter, Instagram, LinkedIn). Recipients were encouraged to share the survey link with their contacts with narcolepsy or idiopathic hypersomnia.

The survey was designed by a multidisciplinary team, including a person with narcolepsy type 1, patient community leaders, clinicians, and researchers. The survey was composed of 30 questions, using both multiple-choice and free-response formats (Supplementary material). Respondents were asked about their diagnosis (narcolepsy type 1, narcolepsy type 2, idiopathic hypersomnia), time from symptom onset to diagnosis, ease or difficulty in adjusting to their diagnosis, whether or not they knew someone with the same diagnosis, the impact of knowing another person with the same diagnosis, methods of communicating with other people with their diagnosis and topics discussed, and sources of support. Open-ended questions provided the opportunity for respondents to describe unmet needs, including concerns or uncertainties about their condition and how they could have been better supported in adjusting to their diagnosis.

Survey respondents voluntarily and anonymously registered their responses using Google Forms. Adaptive questioning was utilized, and responses to prior items determined subsequent items to be completed. All survey questions were optional, and respondents could revise any response prior to final submission.

This survey met the regulatory criteria for exempted projects (45 CFR § 46.104 [d] [4]) and received an exemption from the institutional review board WCG IRB (Princeton, NJ, USA). Respondents did not sign an informed consent form; the fact that they voluntarily completed the survey served as their consent to participate. No financial incentives were provided for survey completion.

2.2. Data analysis

Survey responses were captured electronically, and duplicate submissions (ie, those with identical responses to all items) were removed manually. In October 2023, an independent consultant with experience in biostatistics conducted data analyses using R software [24]. Responses were summarized using descriptive statistics. For some items, responses were rated using a Likert scale rating from 1 to 5, with verbal anchors (eg, very easy, very difficult). Multivariable regression analysis was conducted to assess predictors of feeling supported in adjusting to life with narcolepsy or hypersomnia, and all assumptions were met. Factors considered included gender, age, race, geographic region, marital status, diagnosis, and knowing someone with the same diagnosis. For the purposes of this survey, knowing someone with the same condition was defined as “such that you could communicate with them personally” and felt “comfortable” doing so. Adjusted odds ratios, corresponding 95 % confidence intervals, and p values were calculated for each potential predictor.

3. Results

3.1. Participants

Of 1352 responses received, 44 duplicates were removed, yielding a total of 1308 individuals who completed the survey (Table 1).

Table 1
Demographic characteristics

Characteristic	Overall (n = 1308)	Narcolepsy type 1 (n = 633)	Narcolepsy type 2 (n = 375)	Idiopathic hypersomnia (n = 300)
Age group, y, n (%)				
18–20 y	87 (6.7)	54 (8.5)	18 (4.8)	15 (5.0)
21–30 y	414 (31.7)	204 (32.2)	111 (29.6)	99 (33.0)
31–40 y	398 (30.4)	180 (28.4)	122 (32.5)	96 (32.0)
41–50 y	212 (16.2)	101 (16.0)	63 (16.8)	48 (16.0)
51–60 y	124 (9.5)	59 (9.3)	38 (10.1)	27 (9.0)
≥61 y	72 (5.5)	35 (5.5)	23 (6.1)	14 (4.7)
Not available	1 (0.1)	0	0	1 (0.3)
Gender, n (%)				
Female	1147 (87.7)	557 (88.0)	319 (85.1)	271 (90.3)
Male	125 (9.6)	63 (10.0)	43 (11.5)	19 (6.3)
Nonbinary, transgender, or gender fluid	29 (2.2)	12 (1.9)	9 (2.4)	8 (2.7)
Not available	7 (0.5)	1 (0.2)	4 (1.1)	2 (0.7)
Race, n (%)				
Asian	19 (1.5)	10 (1.6)	6 (1.6)	3 (1.0)
Black or African	36 (2.8)	19 (3.0)	13 (3.5)	4 (1.3)
American White	1162 (88.8)	548 (86.6)	335 (89.3)	279 (93.0)
Other ^a	11 (0.8)	5 (0.8)	4 (1.1)	2 (0.7)
Multiracial	22 (1.7)	13 (2.1)	5 (1.3)	4 (1.3)
Not available	58 (4.4)	38 (6.0)	12 (3.2)	8 (2.7)
Marital status, n (%)				
Married	503 (38.5)	226 (35.7)	163 (43.5)	114 (38.0)
Not married	791 (60.5)	399 (63.0)	207 (55.2)	185 (61.7)
Not available	14 (1.1)	8 (1.3)	5 (1.3)	1 (0.3)
Region, n (%)				
United States	857 (65.5)	375 (59.2)	262 (69.9)	220 (73.3)
Rest of world	444 (33.9)	252 (39.8)	113 (30.1)	79 (26.3)
Africa	5 (0.4)	4 (0.6)	1 (0.3)	0
Asia/ Middle East	9 (0.7)	8 (1.3)	0	1 (0.3)
Australia/ New Zealand	154 (11.8)	71 (11.2)	57 (15.2)	26 (8.7)
Canada	62 (4.7)	36 (5.7)	16 (4.3)	10 (3.3)
Central America/ South America/ Caribbean	13 (1.0)	5 (0.8)	5 (1.3)	3 (1.0)
Europe	201 (15.4)	128 (20.2)	34 (9.1)	39 (13.0)
Not available	7 (0.5)	6 (0.9)		1 (0.3)

^a Includes American Indian or Alaska Native, Indigenous Australian/Aborigine, Maori, and Native Hawaiian or other Pacific Islander.

Respondents reported a diagnosis of narcolepsy type 1 (48.4 %),

narcolepsy type 2 (28.7 %), or idiopathic hypersomnia (22.9 %). A wide range of ages was represented, with 68.7 % of respondents ≤40 years of age; 87.7 % were female, 38.5 % were married, and 88.8 % were White. Respondents were from 38 different countries, with 65.5 % from the United States, 15.4 % from Europe, and 11.8 % from Australia/New Zealand.

3.2. Delayed diagnosis

Diagnostic delay was common among respondents, with more than one-third reporting diagnosis ≥10 years after symptom onset; only one-quarter reported being diagnosed within 3 years (Fig. 1). Time from symptom onset to diagnosis was shorter in respondents with narcolepsy type 1 (≤3 years: 29.2 %; ≥10 years: 35.2 %) versus those with narcolepsy type 2 (≤3 years: 17.9 %; ≥10 years: 38.9 %) or idiopathic hypersomnia (≤3 years: 17.7 %; ≥10 years: 44.7 %).

3.3. Knowing someone with the same diagnosis

At the time of diagnosis, most respondents (91.1 %) did not know anyone with the same diagnosis (Fig. 2). Almost all respondents with idiopathic hypersomnia (96.0 %) reported this, while 88.8 % of respondents with narcolepsy type 1 and 91.2 % of respondents with narcolepsy type 2 reported not knowing anyone with the same diagnosis. Rates were similar for respondents in the US (91.0 %) and rest-of-world (91.2 %) groups as well as across age groups (18–30 years old: 89.8 %; 31–50 years old: 92.1 %; 50+ years old: 91.3 %).

After diagnosis, many years typically elapsed before respondents met another person with the same diagnosis, if they met anyone at all. At the time of the survey, 486 respondents (37.2 %) still did not know anyone with their diagnosis who they could communicate with personally and felt comfortable talking to, and for 17.7 % of respondents, it took more than 3 years to meet someone with the same diagnosis (Fig. 3). The percentage of respondents who still had not met someone was particularly high among people with idiopathic hypersomnia (55.7 %) relative to narcolepsy type 1 (29.7 %) or type 2 (34.9 %). Rates were similar for respondents in the US (36.8 %) and rest-of-world (37.8 %) groups as well as across age groups (18–30 years old: 37.3 %; 31–50 years old: 36.7 %; 50+ years old: 38.8 %).

Of respondents who indicated knowing someone with the same diagnosis (at the time of the survey), 80.7 % reported this had a positive or very positive impact, a finding that was consistent across all diagnostic groups (Fig. 4).

3.4. Adjustment to diagnosis and perceived social support

Most respondents (67.6 %) indicated that adjusting to life with their

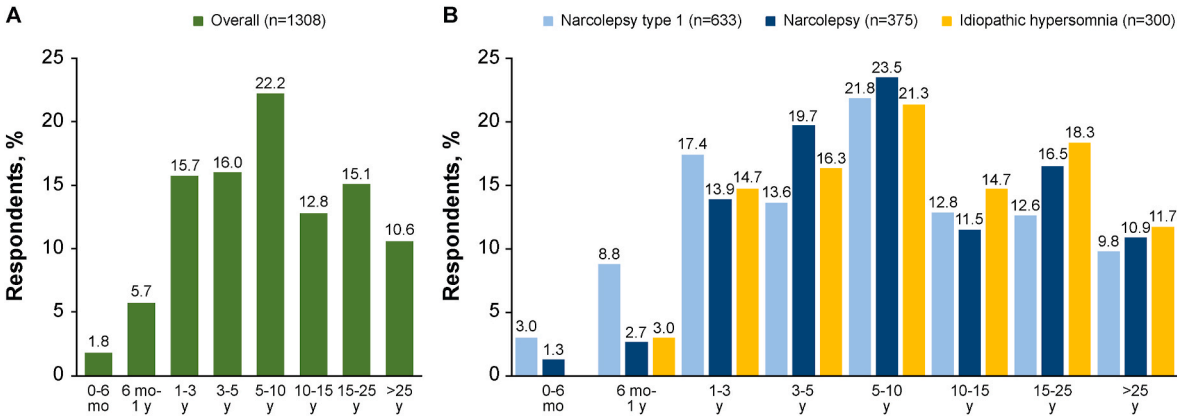


Fig. 1. Time from symptom onset to diagnosis overall (A) and by diagnosis (B). Data missing for 1 respondent.

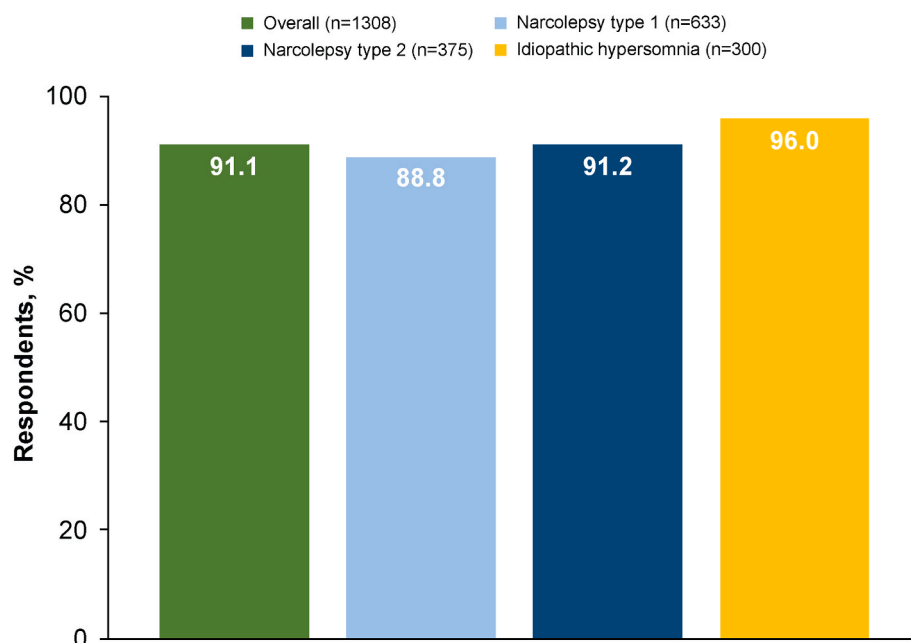


Fig. 2. At time of diagnosis of narcolepsy or idiopathic hypersomnia, proportion of respondents who did not know another person with the same diagnosis. Data missing for 1 respondent.

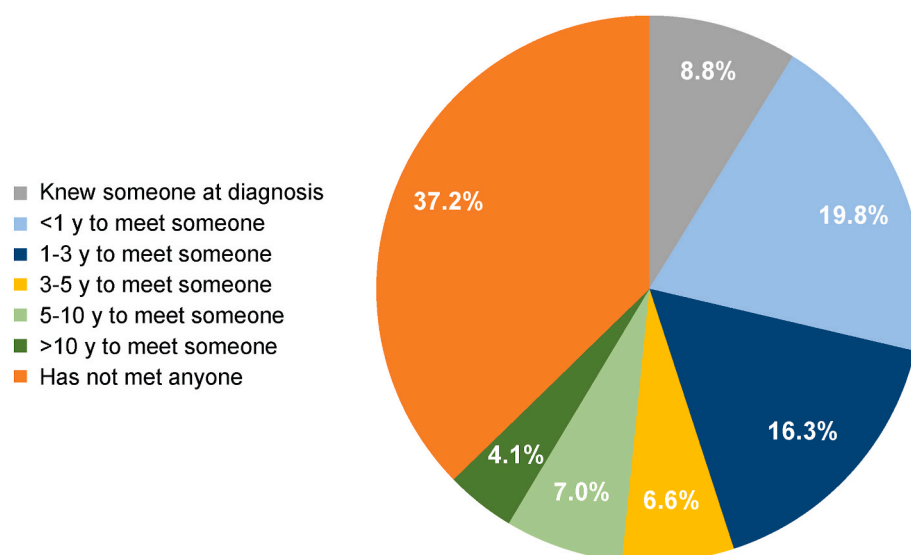


Fig. 3. Meeting another person with the same diagnosis: proportion of respondents who knew someone at diagnosis, met someone between diagnosis and survey (categorized by time to meeting someone), or have not met anyone. Data missing for 3 respondents.

diagnosis was difficult or very difficult (Fig. 5). Adjustment was rated as more difficult by respondents with idiopathic hypersomnia relative to those with narcolepsy. Overall, only 32.3 % of respondents felt supported in adjusting to life with narcolepsy or idiopathic hypersomnia (Fig. 6).

3.5. Associations with perceived social support

Regression analysis found that knowing someone with the same diagnosis (at the time of the survey) was significantly associated with perceived support (Fig. 7). Younger age (18–30 years) and being married were also associated with greater perceived support. A diagnosis of narcolepsy type 2 or idiopathic hypersomnia was associated with less perceived support than a diagnosis of narcolepsy type 1. Living outside the United States was also associated with lower perceived support.

Individuals who identified as nonbinary, transgender, or gender fluid had a trend toward feeling less supported relative to individuals who identified as female ($p = 0.08$); however, the limited representation of the former groups makes this interpretation difficult.

Respondents identified family (57.4 %), friends (49.6 %), narcolepsy/sleep specialists (49.3 %), significant others (47.5 %), and other people with the same diagnosis (40.1 %) as meaningful sources of support. Excluding the 486 survey respondents who did not know another person living with their condition, 63.4 % considered others with their diagnosis as a meaningful source of support. The most common ways for respondents to find others with the same diagnosis were online resources: social media (60.7 %) and online support group (45.4 %). Older respondents (50+ years old) were less likely to find others using online sources: 18–30 years old, 78.2 %; 31–50 years old, 82.9 %; 50+ years old: 65.8 %. Many respondents (31.2 % overall) found others with their

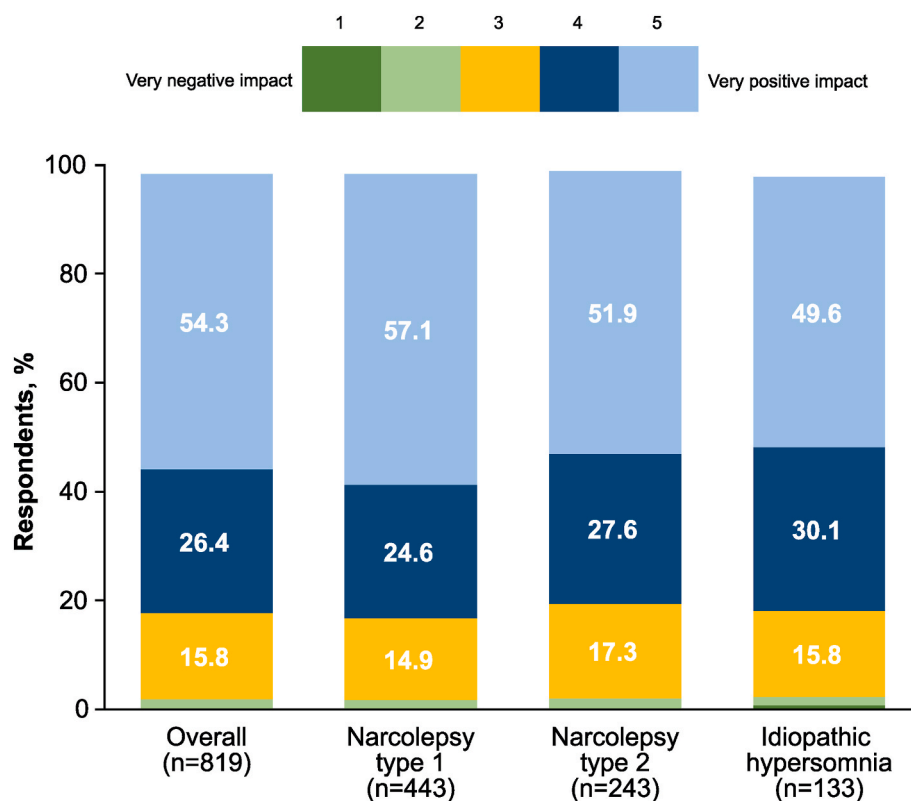


Fig. 4. Impact of knowing someone with the same diagnosis.

Excludes 486 respondents who reported not knowing another person with the same diagnosis; data missing for 15 respondents. Survey question was “Overall, how impactful has knowing other people with the same diagnosis been for you?”

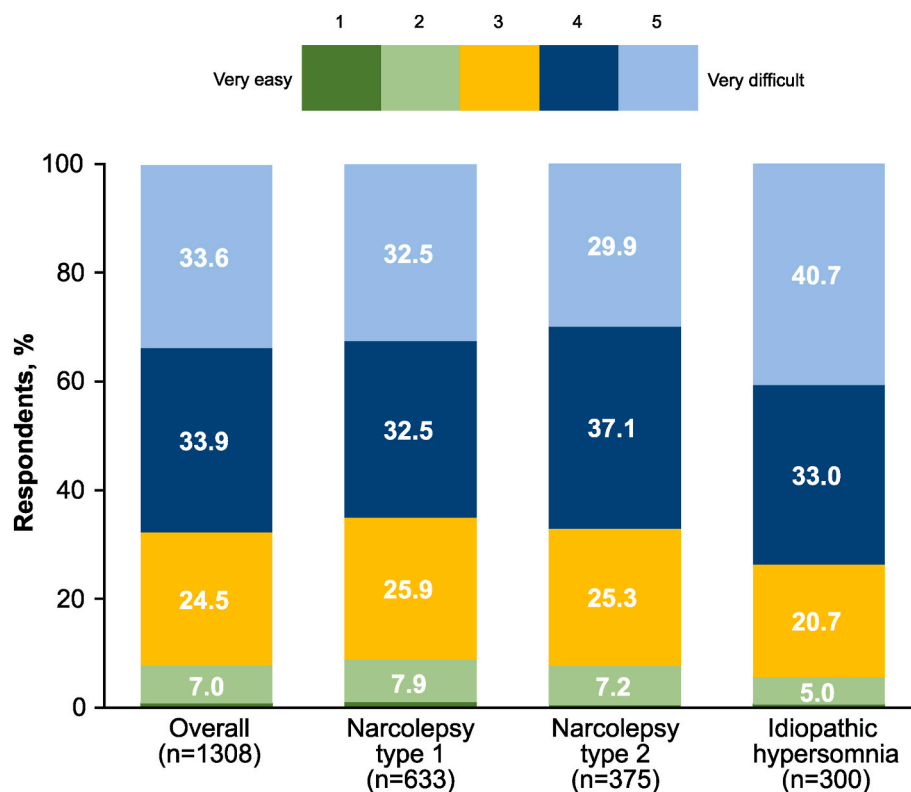


Fig. 5. Level of ease in adjusting to living with narcolepsy or idiopathic hypersomnia.

Survey question was “From your CURRENT perspective, how easy or difficult has it been to adjust to living with narcolepsy or idiopathic hypersomnia?”

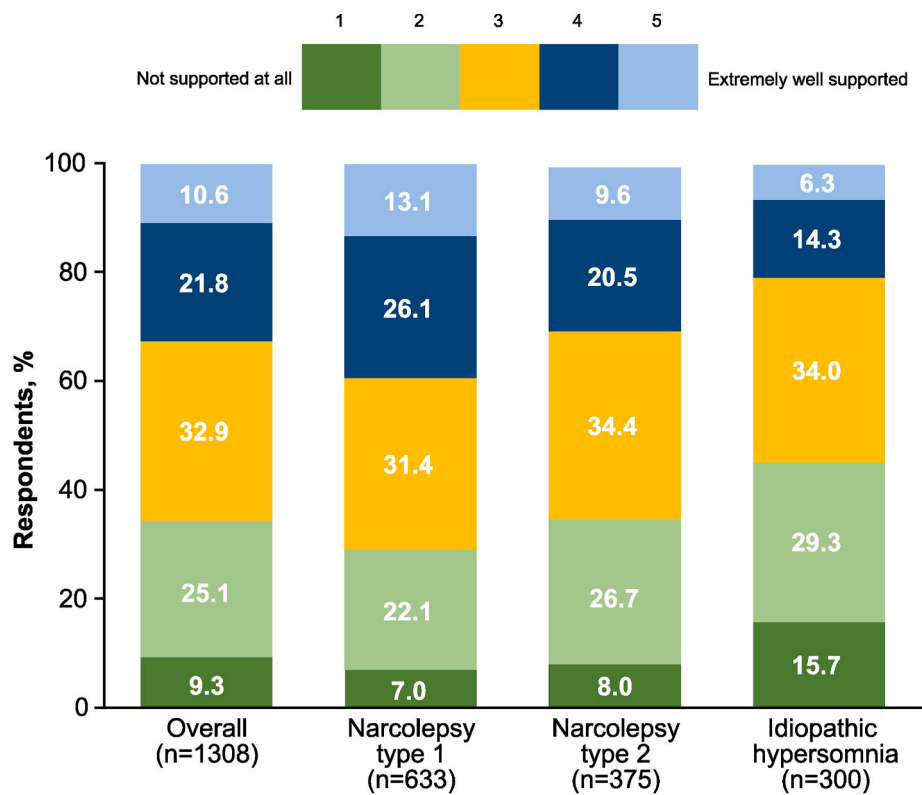


Fig. 6. Perceived support in adjusting to life with narcolepsy or idiopathic hypersomnia. Data missing for 6 respondents. Feeling supported defined as a response of 4 or 5. Survey question was “Overall, how well supported have you felt in adjusting to life with narcolepsy or idiopathic hypersomnia?”

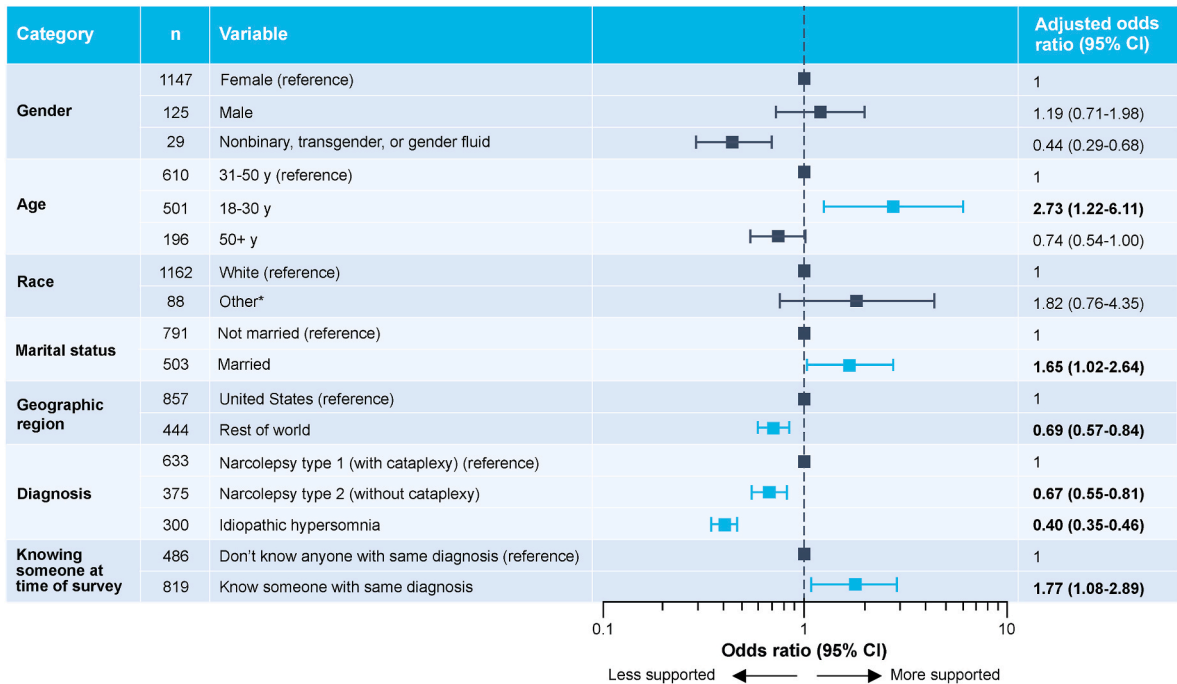


Fig. 7. Multivariable regression analysis of potential predictors of perceived support. Feeling supported was defined as a response of 4 or 5. Values in bold represent statistical significance. CI = confidence interval. *Included American Indian or Alaska Native, Asian, Black or African American, Indigenous Australian/Aborigine, Maori, Native Hawaiian or other Pacific Islander, and multiracial.

diagnosis via programming, conferences, or events sponsored by nonprofit organizations. Social media, the most common method for communicating with others with the same diagnosis, was used by 77.6 %

of respondents. Respondents discussed a wide range of topics with others who have the same diagnosis, including effects of the disease on daily life, treatment options, and lifestyle adjustments (Table 2). Online

Table 2

Topics discussed with other people with narcolepsy or idiopathic hypersomnia.

Topic	Respondents (n = 808)
Treatment options and side effects	652 (80.7)
Tips and tricks for daily living	569 (70.4)
Understanding the diagnosis and symptoms	565 (69.9)
Lifestyle adjustments (sleep habits, diet, fitness)	562 (69.6)
Naps	517 (64.0)
Relationships (family, friends, significant others)	470 (58.2)
Work/school accommodations	461 (57.1)
Communicating about your diagnosis	450 (55.7)
Mental health	416 (51.5)
Health care providers specializing in narcolepsy and idiopathic hypersomnia	388 (48.0)
Driving considerations	369 (45.7)
Finding more resources, organization, or support groups	331 (41.0)
Scientific advancements and clinical trials	331 (41.0)
Treatment access/medical insurance issues	290 (35.9)
Managing additional medical conditions	284 (35.1)
Pregnancy/parenting	255 (31.6)
Media and entertainment (books, movies, videos, blogs)	234 (29.0)
Social security disability	192 (23.8)

sources were widely considered to be helpful resources by respondents: websites, blogs, or articles, 62.4 %; social media, 56.4 %; online support groups, 47.0 %.

4. Discussion

To our knowledge, this is the first study to assess the peer-to-peer connections of people with narcolepsy or idiopathic hypersomnia. At the time of diagnosis, very few respondents knew another person with the same disorder. This is likely due, in part, to narcolepsy and idiopathic hypersomnia being rare conditions with often invisible symptoms. Also, due to stigma, many people may choose not to disclose their condition to others, so people may not realize that they know another person living with the same condition.

For many people, it took several years to have a meaningful connection with someone who shared the same diagnosis. At the time of the survey, one-third of respondents still did not know anyone else with their same condition. This finding is striking because it illustrates that people are not connected with others who have the same conditions, despite existing resources to support these connections. We believe this is, in part, due to underrecognition in clinical practice of patients' social experiences and unmet needs.

Knowing someone with narcolepsy or idiopathic hypersomnia had a positive impact for respondents and was significantly associated with feeling supported in adjusting to the diagnosis. Connecting with another person with the same diagnosis may help people feel less socially isolated. It also may provide pragmatic information and strategies for managing the medical, social, and internal aspects of living with these conditions. This aligns with literature indicating that peer-to-peer connections and role models can provide ongoing support and education, help improve access to care and treatment adherence, and help address psychosocial challenges [25,26]. Studies of peer support have identified potential benefits in disease management among people with diabetes [27] and sleep apnea [28].

The findings of this survey do not provide an explanation for the association between younger age and greater perceived support, as the percentage of respondents who know another person with their diagnosis was similar across age groups. Perceived support was greater in married versus unmarried respondents; however, because the marriage question allowed only a dichotomous (yes/no) response, the impact of being in a long-term relationship outside marriage is not known.

While the need for social support was evident across diagnostic groups in this survey, difficulty adjusting to their diagnosis was greater and perceived social support was lower in respondents with idiopathic

hypersomnia relative to those with narcolepsy. This may be due, in part, to relatively less clinical and scientific understanding of idiopathic hypersomnia, lack of public recognition, and fewer approved treatment options. People with narcolepsy type 2 or idiopathic hypersomnia, respondents living outside the US, and those who did not identify as either male or female reported lower perceived social support. Further research should explore and address these differences in social experiences. There is also a need for additional studies to assess social support in patients with narcolepsy or idiopathic hypersomnia, and to integrate social support interventions into clinical care to improve adjustment to diagnosis.

Health care providers can assist their patients with narcolepsy or idiopathic hypersomnia in finding resources that provide support and facilitate adjustment to their diagnosis. We recommend health care providers ask patients, "Do you know someone living with the same condition?" as a first step toward assessing unmet social needs and opportunities for connection. While health care providers may be hesitant to facilitate introductions between their patients due to privacy considerations (eg, Health Insurance Portability and Accountability [HIPAA] restrictions in the United States), connecting patients with nonprofit patient advocacy organizations protects privacy concerns and allows patients to access a variety of resources and support services. Nonprofit organizations help people find others with the same diagnosis, along with providing education, resources, and support for patients, loved ones, and caregivers.

Strengths of this study include the large number of respondents, the inclusion of people with idiopathic hypersomnia (which is less studied than narcolepsy), and the wide range of ages and geographic regions represented. Another strength of this study is the multidisciplinary collaboration, which fostered a uniquely patient-centered lens for exploring an often-overlooked aspect of living with narcolepsy or idiopathic hypersomnia. Research teams aiming to better understand patients' experiences and unmet needs should involve patients and patient community leaders from the beginning, including in study conceptualization and design, in keeping with the principles of community-based participatory research [29].

Study limitations include self-reported diagnosis, which was not confirmed in medical records. It was not possible to analyze relationships between social support/connection and symptomatology because the survey did not collect information about symptoms or symptom severity. The survey was conducted in 2021 during the COVID pandemic, and the potential effects on responses to a survey assessing social support are not known. While duplicate submissions were removed, it is possible that an individual could have submitted multiple surveys with some variation in the responses. We consider this possibility unlikely, and a worthwhile tradeoff with protecting respondents' anonymity.

Selection bias is inherent to survey research; for example, there was a preponderance of female and White respondents to this survey relative to the known distribution of people with narcolepsy or idiopathic hypersomnia. There were not enough respondents in the non-White race categories to contribute to the regression model in a meaningful way, and sample sizes of respondents from regions outside the United States varied too widely for analyses by geographic region. Research has shown that the prevalence of diagnosed idiopathic hypersomnia is greater in females, but narcolepsy is not known to have sex-based differences in prevalence [23,30]. Future research could further evaluate the importance of social connection in people with racial identities other than White and in men with narcolepsy or idiopathic hypersomnia. Almost all respondents diagnosed with narcolepsy type 2 or idiopathic hypersomnia came from the United States, which may reflect regional differences in the use of the Multiple Sleep Latency Test and/or extended-duration polysomnography in the evaluation of sleep disorders [23,31]. Also, the survey was distributed through the communication channels of a patient advocacy organization, so respondents may have been more likely to have higher levels of patient activation and be

more connected with existing resources and social support than the broader population of people living with narcolepsy or idiopathic hypersomnia. Future research could explore the motivating factors for seeking social support.

In conclusion, in this survey with more than a thousand respondents, knowing another person with the same diagnosis was associated with greater perceived support in people with narcolepsy or idiopathic hypersomnia. Treating clinicians can facilitate support by connecting people with patient advocacy organizations.

CRediT authorship contribution statement

Julie Flygare: Writing – review & editing, Writing – original draft, Visualization, Project administration, Methodology, Funding acquisition, Formal analysis, Conceptualization. **Lauren Oglesby:** Writing – review & editing, Writing – original draft, Visualization, Project administration, Methodology, Formal analysis, Data curation, Conceptualization. **Sairam Parthasarathy:** Writing – review & editing, Formal analysis. **Michael J. Thorpy:** Writing – review & editing, Formal analysis. **Emmanuel Mignot:** Writing – review & editing, Formal analysis. **Eileen B. Leary:** Methodology, Formal analysis, Writing – review & editing. **Anne Marie Morse:** Formal analysis, Writing – review & editing.

Declaration of competing interest

The authors declare the following financial interests/personal relationships which may be considered as potential competing interests: Julie Flygare reports financial support was provided by Avadel Pharmaceuticals PLC. Julie Flygare reports financial support was provided by Harmony Biosciences. Julie Flygare reports a relationship with Alkermes that includes: consulting or advisory. Julie Flygare reports a relationship with Harmony Biosciences that includes: consulting or advisory. Julie Flygare reports a relationship with Project Sleep that includes: employment. Lauren Oglesby reports a relationship with Project Sleep that includes: employment. Sairam Parthasarathy reports a relationship with Jazz Pharmaceuticals Inc that includes: consulting or advisory. Sairam Parthasarathy reports a relationship with AbbVie Inc that includes: consulting or advisory. Sairam Parthasarathy reports a relationship with Apria Healthcare Group LLC that includes: consulting or advisory. Sairam Parthasarathy reports a relationship with Verily Life Sciences Inc that includes: funding grants. Sairam Parthasarathy reports a relationship with Philips, Inc that includes: funding grants. Sairam Parthasarathy reports a relationship with Sommetics, Inc that includes: funding grants. Sairam Parthasarathy reports a relationship with WHOOP, Inc that includes: funding grants. Sairam Parthasarathy reports a relationship with Regeneron Pharmaceuticals Inc that includes: funding grants. Sairam Parthasarathy reports a relationship with US Biotest, Inc that includes: funding grants. Sairam Parthasarathy reports a relationship with The National Institutes of Health that includes: funding grants. Sairam Parthasarathy reports a relationship with US Department of Defense that includes: funding grants. Sairam Parthasarathy reports a relationship with Pima County Health Department that includes: funding grants. Sairam Parthasarathy reports a relationship with Arizona Commerce Authority that includes: funding grants. Sairam Parthasarathy reports a relationship with Sergey Brin Family Foundation that includes: funding grants. Michael J. Thorpy reports a relationship with Axsome Therapeutics Inc that includes: consulting or advisory. Michael J. Thorpy reports a relationship with Balance Therapeutics that includes: consulting or advisory. Michael J. Thorpy reports a relationship with Eisai Co, Ltd that includes: consulting or advisory. Michael J. Thorpy reports a relationship with Avadel Pharmaceuticals that includes: consulting or advisory. Michael J. Thorpy reports a relationship with Harmony Biosciences that includes: consulting or advisory. Michael J. Thorpy reports a relationship with Jazz Pharmaceuticals Inc that includes: consulting or advisory. Michael J. Thorpy reports a

relationship with NLS Pharmaceuticals that includes: consulting or advisory. Michael J. Thorpy reports a relationship with Suven Life Sciences Limited that includes: consulting or advisory. Michael J. Thorpy reports a relationship with Takeda Pharmaceutical Company Limited that includes: consulting or advisory. Emmanuel Mignot reports a relationship with Centessa Pharmaceuticals that includes: consulting or advisory. Emmanuel Mignot reports a relationship with Takeda Pharmaceutical Company Limited that includes: consulting or advisory. Emmanuel Mignot reports a relationship with Harmony Biosciences that includes: consulting or advisory and funding grants. Emmanuel Mignot reports a relationship with Merck & Co Inc that includes: consulting or advisory. Emmanuel Mignot reports a relationship with National Institutes of Health that includes: funding grants. Emmanuel Mignot reports a relationship with Alkermes that includes: funding grants. Emmanuel Mignot reports a relationship with Apple that includes: funding grants. Emmanuel Mignot reports a relationship with Avadel Pharmaceuticals that includes: funding grants. Emmanuel Mignot reports a relationship with Eisai Co, Ltd that includes: funding grants. Emmanuel Mignot reports a relationship with Jazz Pharmaceuticals Inc that includes: funding grants. Emmanuel Mignot reports a relationship with The ResMed Foundation that includes: funding grants. Eileen B. Leary reports a relationship with Axsome Therapeutics that includes: employment. Anne Marie Morse reports a relationship with Avadel Pharmaceuticals that includes: consulting or advisory and speaking and lecture fees. Anne Marie Morse reports a relationship with Eisai Co, Ltd that includes: consulting or advisory. Anne Marie Morse reports a relationship with Harmony Biosciences that includes: consulting or advisory and speaking and lecture fees. Anne Marie Morse reports a relationship with Jazz Pharmaceuticals that includes: consulting or advisory, funding grants, and speaking and lecture fees. Anne Marie Morse reports a relationship with NLS Pharmaceuticals that includes: consulting or advisory. Anne Marie Morse reports a relationship with Alkermes that includes: consulting or advisory. Anne Marie Morse reports a relationship with Takeda Pharmaceutical Company Limited that includes: consulting or advisory and speaking and lecture fees. Anne Marie Morse reports a relationship with Neura Health that includes: consulting or advisory. Anne Marie Morse reports a relationship with National Institutes of Health that includes: funding grants. Anne Marie Morse reports a relationship with UCB Pharmaceuticals that includes: funding grants. Anne Marie Morse reports a relationship with The ResMed Foundation that includes: funding grants. Anne Marie Morse reports a relationship with Coverys Community Healthcare Foundation that includes: funding grants. Anne Marie Morse reports a relationship with Geisinger Health Plan that includes: funding grants. Anne Marie Morse reports a relationship with DAMM Good Sleep, LLC that includes: employment. Sairam Parthasarathy, co-author, receives royalties from UpToDate, Inc., for writing a book chapter.

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Appendix A. Supplementary data

Supplementary data to this article can be found online at <https://doi.org/10.1016/j.sleep.2024.11.013>.

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